

SALMAX™ CR

Tablet 4mg

PRODUCT DESCRIPTION

Salmax CR Tablet 4mg are white, hexagonal-shaped tablet embossed with 4 on one side and plain on the other side.

COMPOSITION

Each Salmax CR Tablet 4mg contains 4 mg of Salbutamol as sulphate in a controlled release delivery system.

Preservative: none

PHARMACODYNAMICS

Salbutamol is a direct-acting sympathomimetic agent with predominantly beta-adrenergic activity and a selective action on beta₂ receptors in bronchial smooth muscle. This preference for beta₂ receptor stimulation results in its bronchodilating action being relatively more prominent than its effect on the heart.

The action of Salbutamol is believed to result from increased production of cyclic adenosine 3', 5'-monophosphate (c-AMP) caused by the activation of enzyme adenyl cyclase which catalyzes the conversion of adenosine triphosphate (ATP) to c-AMP. Increased c-AMP concentrations relax the bronchial smooth muscle and inhibit the release of mediators of immediate hypersensitivity from cells, especially mast cells. Salbutamol relaxes bronchial smooth muscle in the lungs, thereby relieving bronchospasm, increasing vital capacity, decreasing residual volume and reducing airway resistance.

PHARMACOKINETICS

Salbutamol is readily absorbed from the gastro-intestinal tract. It undergoes first-pass metabolism in the liver and possibly in the gut wall, producing an inactive sulphate conjugate as the main metabolite. It is rapidly excreted in the urine as metabolites and unchanged drug; there is some excretion in the faeces. Following oral administration, the onset of action is within 30 minutes, with a peak effect between 5 to 6 hours after the dose. The plasma half-life of Salbutamol is about 2 to 7 hours. For controlled-release preparations, the duration of action is up to 12 hours.

INDICATIONS

Salbutamol is indicated for the symptomatic treatment of bronchial asthma. It is also indicated for the treatment of reversible bronchospasm associated with bronchitis, pulmonary emphysema, bronchiectasis and other obstructive pulmonary disease.

SIDE EFFECTS/ADVERSE REACTIONS

Salbutamol may cause fine tremor of skeletal muscle (particularly the hands), palpitations and muscle cramps. Slight tachycardia, tenseness, headaches and peripheral

vasodilatation have been reported after large doses. Hypokalaemia may be induced by higher-than recommended doses of Salbutamol. Hypersensitivity reactions including angioedema, urticaria, bronchospasm, hypotension and collapse have been reported very rarely.

PRECAUTIONS/WARNINGS

Salbutamol should be used with caution in patients with hyperthyroidism and thyrotoxicosis. Great care is also needed in patients with cardiovascular disease such as ischaemic heart disease, arrhythmia; occlusive vascular disorders including arteriosclerosis and hypertension.

Oral administration of Salbutamol during labour has been reported to inhibit uterine contractions.

Unnecessary administration of Salbutamol during the first trimester of pregnancy is undesirable. Administration of Salbutamol during pregnancy should only be considered if the expected benefit to the mother is greater than any possible risk to the foetus.

It is not known whether Salbutamol is distributed into breast milk and its use in nursing mothers is not recommended.

Tocolysis: Serious adverse reactions including death have been reported after administration of terbutaline/salbutamol to women in labor. In the mother, these include increased heart rate, transient hyperglycaemia, hypokalaemia, cardiac arrhythmias, pulmonary oedema and myocardial ischaemia. Increased fetal heart rate and neonatal hypoglycaemia may occur as a result of maternal administration.

As maternal pulmonary oedema and myocardial ischaemia have been reported during or following premature labour in patients receiving beta₂ – agonists, careful attention should be given to fluid balance and cardio-respiratory function, including ECG monitoring. If signs of pulmonary oedema and myocardial ischaemia develop, discontinuation of treatment should be considered.

Due to the risk of pulmonary oedema and myocardial ischaemia that has been observed during the use of beta₂-agonists in the treatment of premature labour, before these products are given to any patient with known heart disease, an adequate assessment of the patient's cardiovascular status should be made by a physician experienced in cardiology.

CONTRAINDICATION

Salmax tablets are contra-indicated in patients who are hypersensitive to any of their components.

Although Salbutamol tablets are occasionally used in the management of uncomplicated premature labour, it should not be used for threatened abortion during the first or second trimesters of pregnancy.

DRUG INTERACTIONS

Concurrent use of tricyclic antidepressants and Salbutamol may potentiate the action of Salbutamol on the vascular system.

Beta-adrenergic blocking agents may antagonise the effects of Salbutamol.

Concurrent use of digitalis glycosides with Salbutamol may increase the risk of cardiac arrhythmias.

Monoamine oxidase inhibitors may potentiate the beta-adrenergic stimulating effects of Salbutamol when used concurrently.

An enhanced hypokalaemic effect may occur during concomitant treatment of Salbutamol with theophylline, corticosteroids and diuretics.

Co-administration of diuretics as antihypertensives with Salbutamol, may reduce the antihypertensive effects.

RECOMMENDED DOSAGE, DOSAGE SCHEDULE

Adult: Oral, 4 or 8 mg every twelve hours. The usual prescribing limits is up to 32 mg per day.

Elderly: A lower dose should be given initially. In certain circumstances, such as adults of low body weight, it may be desirable to use a starting dosage of 4 mg every 12 hours and progress to 8 mg every 12 hours according to response.

Paediatric:
Children 3 to 12 years of age - The recommended dose is 4 mg tablet twice daily.

Children 12 years of age and over - *see adult dose*.

Note: Excessive dosing may cause undesirable adverse effects, hence the dosage or frequency of administration should only be increased on medical advice.

Administration
Salmax CR tablets must be swallowed whole with a glass of water and not chewed or crushed.

PREGNANCY AND LACTATION

Unnecessary administration of Salbutamol during the first trimester of pregnancy is undesirable. Administration of Salbutamol during pregnancy should only be considered if the expected benefit to the mother is greater than any possible risk to the foetus.

It is not known whether Salbutamol is distributed into breast milk and its use in nursing mothers is not recommended.

SYMPTOMS AND TREATMENT FOR OVERDOSAGE AND ANTIDOTE(S)

Symptoms of overdose that have been reported include tachycardia, CNS stimulation, tremor, hypokalaemia, and hyperglycaemia. The plasma-potassium concentration and pulse rate have been found to correlate with the plasma concentration of Salbutamol.

The recommended treatments of overdose include reducing dosage or discontinuing medication, supportive therapy, performing gastric lavage and administering a cardioselective beta-adrenergic blocker (e.g. acebutalol, atenolol, metoprolol). Beta-adrenergic blocker should be used with caution because it could induce severe bronchospasm or an asthmatic attack. Symptomatic treatment of adverse effects has proved successful.

PHARMACEUTICAL INFORMATION

Storage conditions

Store below 30°C in a cool dry place.

Protect from moisture and heat.

Keep out of reach of children.

Jauhi ubat-ubatan daripada kanak-kanak.

Route of administration: Oral

Shelf Life

Please refer to outer package.

Pack size / package

Salmax 4 mg CR Tablet Blister packs of 14 x 4 mg tablets in a strip, 4 strips in a pack.

Product Registration Holder:

DUOPHARMA MARKETING SDN. BHD.

Lot No. 2, 4, 6, 8 & 10 Jalan P/7, Section 13,
Bangi Industrial Estate,
43650 Bandar Baru Bangi,
Selangor, Malaysia.

Manufacturer:

DUOPHARMA MANUFACTURING (BANGI) SDN. BHD.

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Added route of administration

Update manufacturer and PRH name and address

Update company logo

Update revision date